Sullivan, A. L., Harris, B., Kulkarni, T., & Weeks, M. (2020, February). *Demystifying disproportionality in autism identification: Prevalence, policy, and best practice*. Symposium at the annual convention of the National Association of School Psychologists, Baltimore, MD.

Demystifying Disproportionality in Autism Identification: Prevalence, Policy, and Best Practice

Amanda L. Sullivan, PhD
Mollie Weeks, MA
Tara Kulkarni, MA
University of Minnesota

Bryn Harris, PhD
University of Colorado Denver

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University of Colorado Denver
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The contents of this report were developed in part under a grant from the U.S. Department of Education, # H325D160016. However, those contents do not necessarily represent the policy of the U.S. Department of Education, and you should not assume endorsement by the Federal Government. Project Officer, Sarah J. Allen, Ph.D.

IDEAs that Work
Office of Special Education Programs
U.S. Department of Education

Agenda

Agenda	Guiding Questions	Learner Objectives
Part 1 – Amanda Sullivan	What does disproportionality mean in the context of autism?	Describe patterns and disparities
Part 2 – Tara Kulkarni & Mollie Weeks	What is the role of IDEA? What can we glean from case law?	Understand policy dimensions
Part 3 – Bryn Harris	What are the roles of language and culture? What are the limitations of current practice? What practices should we promote?	Identify principles of culturally and linguistically appropriate assessment

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Amanda L. Sullivan, PhD, University of Minnesota

Autism & Disproportionality

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Special Education's Persistent Tension

"The same program that can separate disadvantaged students from their peers, distinguish them with a stigmatizing label, and subject them to a curriculum or low expectations can also provide additional resources, supports, and services without which they cannot benefit from education…

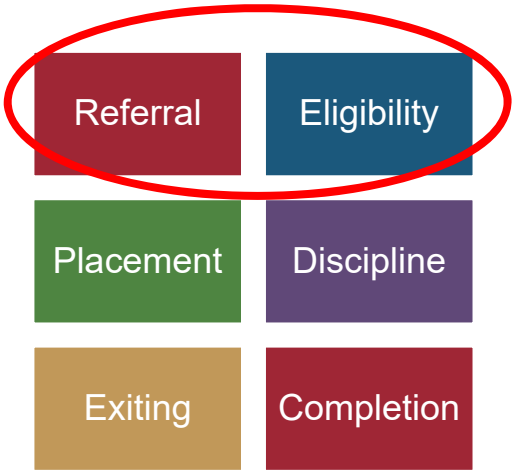


Donovan & Cross, 2002, p. 20

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What Is "Disproportionality"?

- **Group differences** in identification, placement, or other outcomes
  - **Differences in individual's risk** due to sociodemographic group membership
- Common indices: risk, relative risk, composition



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Disproportionality in Context

Disproportionality is inseparable from other educational outcomes and inequities



Sullivan, A. L., & Proctor, S. (2016). The shield or the sword?: Revisiting the debate on racial disproportionality in special education and implications for school psychologists. *School Psychology Forum, 10*, 278-288.

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Are We Identifying the Right Students?

What is disability?

Do we reliably identify disability?

Are educational decisions unbiased?

How do we account for environmental determinants?

… We're not sure.

Donovan & Cross, 2002
Sullivan, A. L., Sadeh, S., & Houri, A. (2019). Are school psychologists' decisions reliable and unbiased? A multi-study experimental investigation. *Journal of School Psychology, 77*, 90-109. https://doi.org/10.1016/j.jsp.2019.10.006

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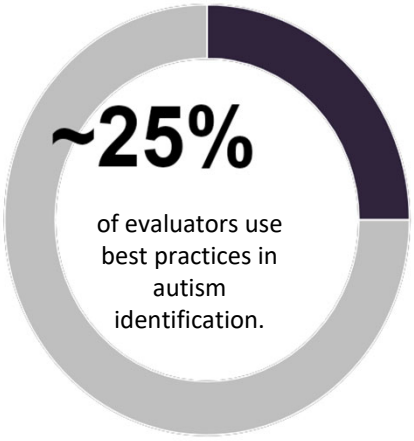
Reliability & Bias in School-based Evaluation

- clinicians frequently have difficulty differentiating ASD from other disorders, particularly for CLD youth
- racial/ethnic minority children who meet diagnostic criteria tend to be identified later in school
- school psychologists may be prone to under-refer and under-identify students who present with characteristics of autism
  - Detection of bias complicated by lack of data-based decisions and general unreliability

Sullivan, A. L., Sadeh, S., & Houri, A. (2019). Are school psychologists' decisions reliable and unbiased? A multi-study experimental investigation. *Journal of School Psychology, 77*, 90-109. https://doi.org/10.1016/j.jsp.2019.10.006

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Challenges to School-based Identification

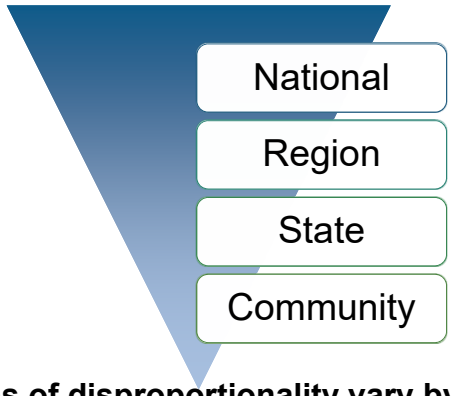


School-based policy, procedures, and practices are **malleable** targets for outreach, policy refinement, and professional learning.

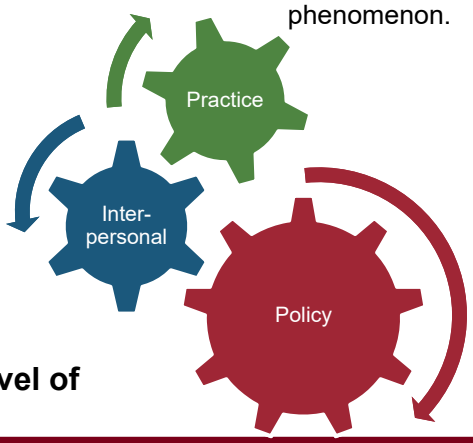
Aiello, Ruble, & Esler, 2017; Akshoomoff, Corsello, & Schmidt, 2006; Luiselli et al., 2001; Wiggins et al., 2006; Williams et al., 2009

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Describing Disproportionality Is Complicated



Disproportionality is a 'multi-determined' phenomenon.



Patterns of disproportionality vary by disability, level of aggregation, and unit of analysis.

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ASD: CDC v. Special Ed

Sex	Clinical Diagnosis	Educational Diagnosis
Male		
Proportion of Total	72.68%	84.15%
Prevalence Among Group	1 in 38	1 in 54
Frequency	3,978	459,341
Female		
Proportion of Total	17.21%	15.85%
Prevalence Among Group	1 in 152	1 in 285
Frequency	942	86,501
Total		
Prevalence	1 in 59	1 in 91
Frequency	5,473	545,842

Race/Ethnic Group	Clinical Diagnosis	Educational Diagnosis
African American		
Proportion of Total	25.50%[a]	14.90%
Prevalence Among Group	1 in 63	1 in 131
Frequency	67,246	77,358
Asian American		
Proportion of Total	5.30%[a]	5.70%
Prevalence Among Group	1 in 127[b]	1 in 118
Frequency	13,903	29,530
Hispanic		
Proportion of Total	19.10%[a]	21.20%
Prevalence Among Group	1 in 71	1 in 159
Frequency	50,258	110,347
White		
Proportion of Total	49.60%[a]	58.20%
Prevalence Among Group	1 in 59	1 in 126
Frequency	130,930	303,075

Barnard-Brak, L. (2019). Educational versus clinical diagnoses of autism spectrum disorder: Updated and expanded findings. *School Psychology Review* 48, (2), 185-189.

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Relative Risk Varies by State

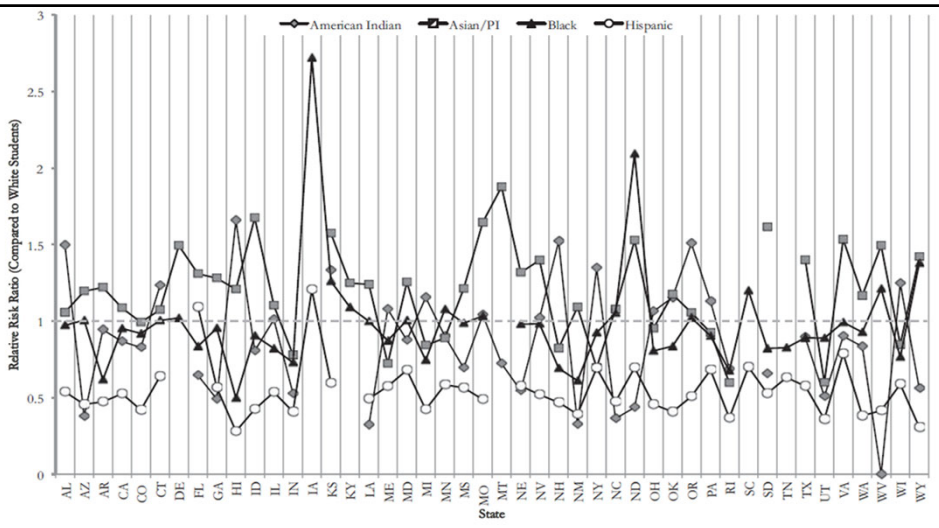
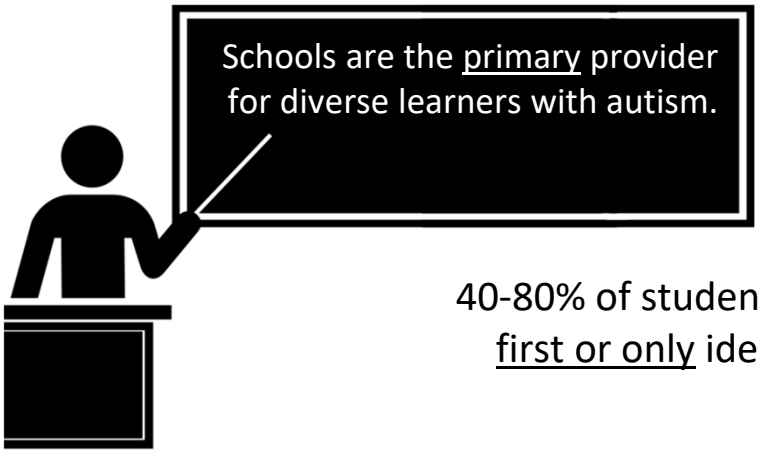


Figure 1. Racial minority groups' relative risk of school-based autism identification varied substantially by state in 2008–2009. The relative risk ratio is a ratio of the prevalence for the minority group and White students. In states

Sullivan, A. L. (2013). School-based autism identification: Prevalence, racial disparities, and systemic correlates. *School Psychology Review*, 42, 298-316.

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Special Education Access Matters



40-80% of students with autism are first or only identified by schools.

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Why Equity in Access Matters for Students with Autism

- Subjectivity of diagnoses/identification
- Differential access to health information and care
- Limited professional capacity to diagnose or identify in diverse populations
- Importance of early intervention and appropriate educational services for learning and wellbeing

Social justice is both a process and a goal that requires action. School psychologists work to ensure the **protection of the educational rights, opportunities**, and well-being of all children, especially those whose **voices** have been muted, identities obscured, or **needs ignored**. Social justice requires **promoting non-discriminatory practices and the empowerment of families and communities**. School psychologists enact social justice through c**ulturally-responsive professional practice and advocacy** to create schools, communities, and systems that **ensure equity and fairness** for all children and youth.

Adopted by the NASP Board of Directors, April 2017

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- ✓ Are we identifying the right students?
- ✓ Are the services and supports they receive as a result beneficial?

The Goal

Fulfilling the spirit of special education:

free appropriate public education

following an **individualized** education plan

in the **least restrictive** environment

using **research-based** practices

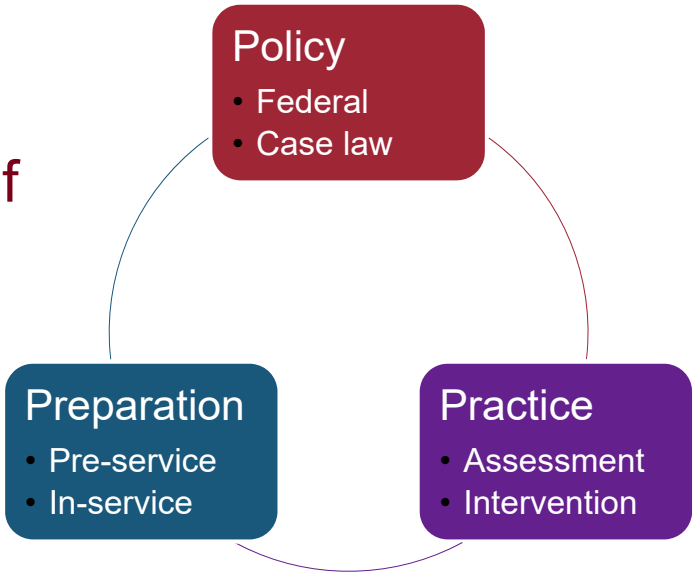
protecting **parent rights**

allowing **due process**

with **zero rejects**

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Interrelated Dimensions of Autism Services



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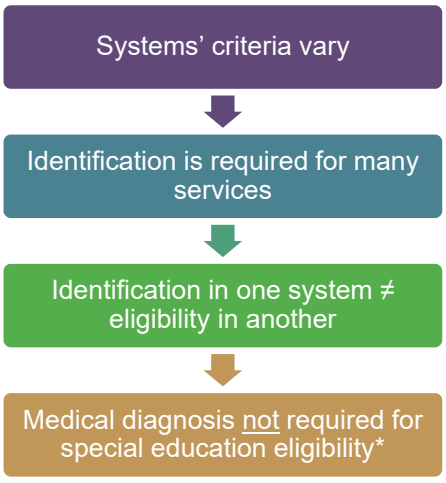
Tara Kulkarni, MA, University of Minnesota

Policy Shaping Autism Identification in Schools

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Autism Classification Systems

- Health service providers:
  - Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition (DSM-5)
  - International Classification of Diseases -10th Revision-Clinical Modification (ICD-1)
- In schools:
  - Federal and state special education law

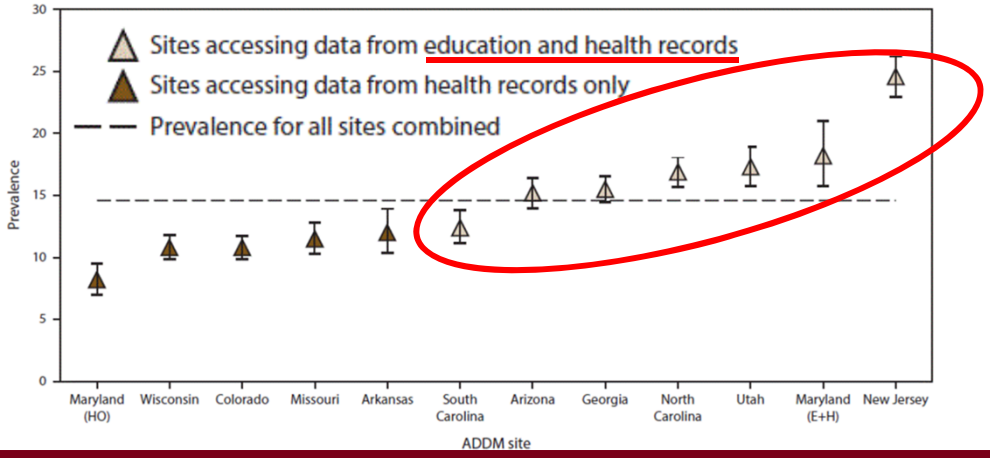


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School Identification Matters

FIGURE 1. Estimated prevalence* of autism spectrum disorder among children aged 8 years — Autism and Developmental Disabilities Monitoring Network, 11 sites, United States, 2012



https://www.cdc.gov/mmwr/volumes/65/ss/ss6503a1.htm

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Autism under IDEA

- (i) Autism means a developmental disability significantly affecting **verbal and nonverbal communication and social interaction**, generally evident before age three, that adversely affects a child's educational performance. Other characteristics often associated with autism are engagement in repetitive activities and stereotyped movements, resistance to environmental change or change in daily routines, and unusual responses to sensory experiences.
- …
- (iii) A child who manifests the characteristics of autism after age three could be identified as having autism if the criteria in paragraph (c)(1)(i) of this section are satisfied.

https://sites.ed.gov/idea/regs/b/a/300.8

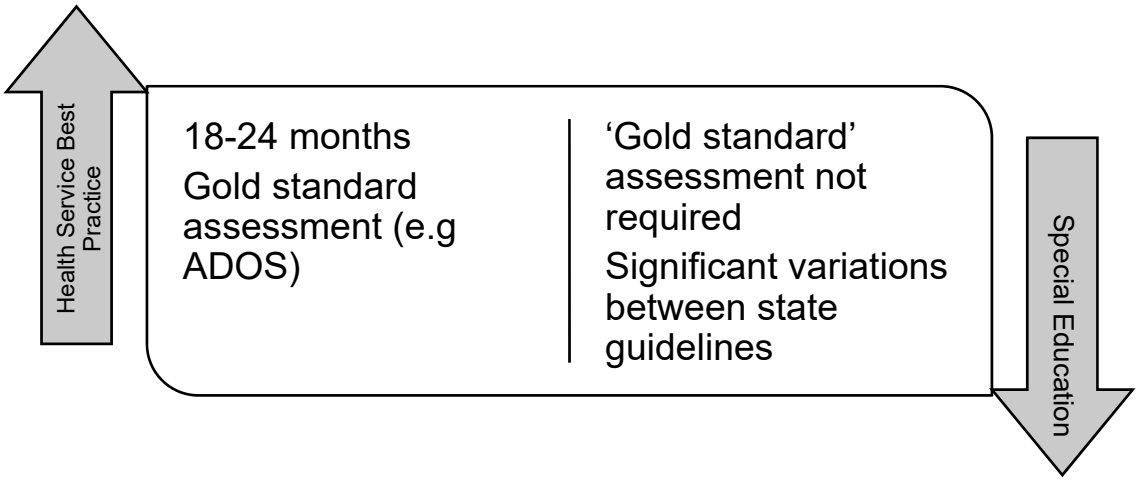
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IDEA Disability Categories

- **autism**
- deaf-blindness
- deafness
- emotional disturbance
- hearing impairment
- intellectual disability
- multiple disabilities
- orthopedic impairment
- other health impairment
- specific learning disability
- speech or language impairment
- traumatic brain injury
- visual impairment

Disproportionality Policy and Autism



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State ASD Criteria Vary

Table 2 Eligibility criteria for ASD identification across states

State	V/H	Med	Neuro	M Dx	Motor	PPE	L/C	SI	Ad. B	IQ	AA	SDH	AR	ASD
AL	X					X	X	X	X					X
AK		X		X		X			X		X	X	X	
AZ														
AR	X	X					X		X	X	X		X	
CA														
CO														X
CT					X		X	X	X	X	X			
DE							X	X						X
FL						X	X	X	X	X		X	X	X
GE						X	X	X	X	X	X	X		X
HA														
ID				X										
IL														
IN					X		X	X	X		X			
IA									X		X	X	X	
KS													X	
KY														
LO	X	X			X	X	X	X	X	X	X	X	X	
ME						X							X	X
MD											X			
MA														
MI							X	X						X
MN							X	X					X	X

Barton, E., Harris, B., Leech, N., Stiff, L., Choi, G. & Joel, T. (2016). an analysis of state autism educational assessment practices and requirements. *Journal of Autism and Developmental Disorders, 46,* 737-748 . doi: 10.1007/s10803-015-2589-0.

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State ASD Rates Vary

Table 1 States ASD prevalence and ASD identification characteristics

State	Prevalence 6–21 (%)	Prevalence 3–5 (%)	Autism in part C	Use DSM	Accept outside eval	Requirements for outside eval	Family input
AL	5.5	6.3			X	X	X
AK	4.2	5.6	X		X	X	X
AZ	3.1	7.2	X				
AR	2.2	5.7	X		X	X	
CA	17.9	9.6	X				
CO	2.9	5.4	X				X
CT	9.9	10.1	X				X
DE	8.5	5.8	X				X
FL	7.2	6.0	X		X	X	X
GE	4.9	7.6	X				X
HA	10.5	7.0	X				
ID	3.8	9.0	X		X	X	
IL	4.8	6.6	X		X	X	
IN	5.1	7.9					X
IA	1.1	1.1	X				X
KS	2.9	5.0	X		X	X	X
KY	2.7	5.2	X				
LO	4.3	5.2	X		X	X	X
ME	11.2	8.4		X	X	X	
MD	6.0	9.6	X				X
MA	13.9	7.9	X		X	X	X
MI	5.9	7.6	X		X	X	
MN	9.5	13.1	X				X

Barton, E., Harris, B., Leech, N., Stiff, L., Choi, G. & Joel, T. (2016). An Analysis of State Autism Educational Assessment Practices and Requirements. Journal of Autism and Developmental Disorders. doi: 10.1007/s10803-015-2589-0.

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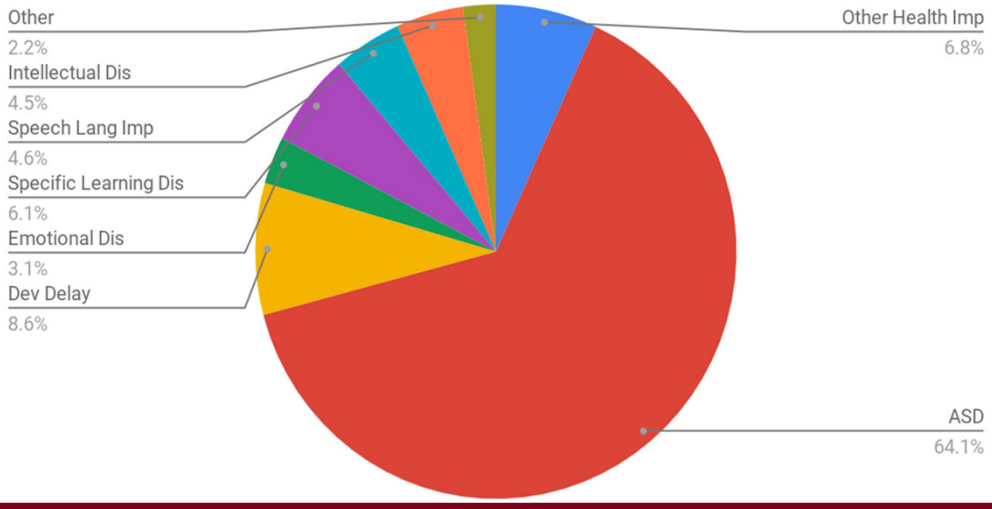
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Diagnostic Substitution

Evaluators fail to recognize ASD or decide that difficulties are more appropriately associated with attributed to another category.

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Percent of Children with a DSM ASD diagnosis receiving special education services (2010)



Rubenstein, E., Daniels, J., Schieve, L. A., Christensen, D. L., Van Naarden Braun, K., Rice, C. E., ... & Lee, L. C. (2018). Trends in special education eligibility among children with autism spectrum disorder, 2002-2010. Public Health Reports, 133(1), 85-92.

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Implications

Special education categories drive services

Schools remain primary provider of Autism services for culturally and linguistically diverse students

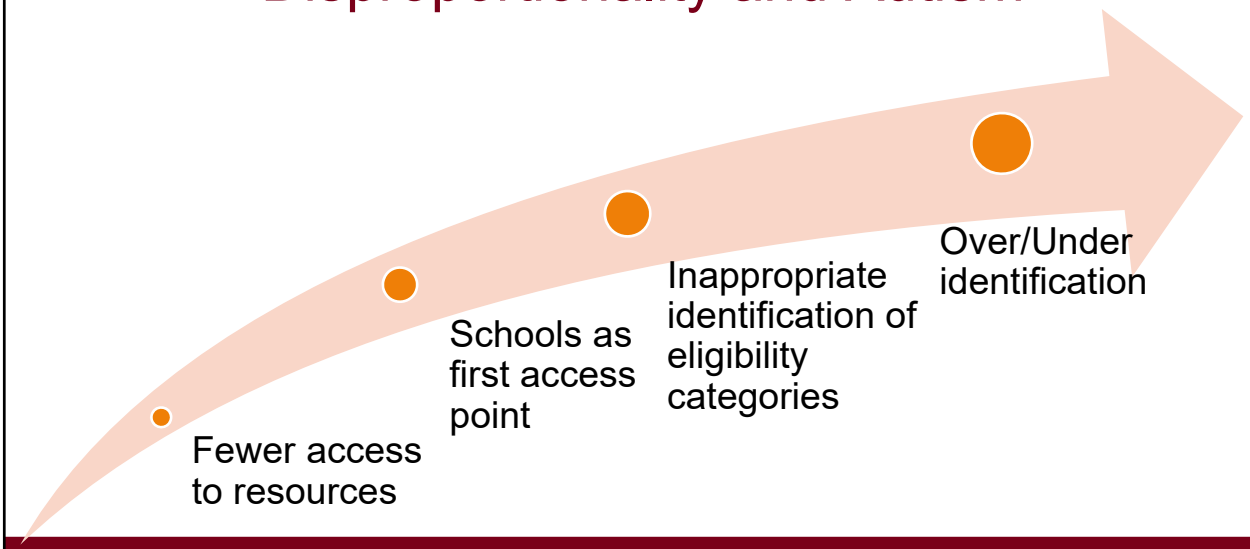
Discrepancy between number of children medically diagnosed with ASD and the number of students receiving special education services

Possibility of under identification

Boswell, K., Zablotsky, B., & Smith, C. (2014). Predictors of autism enrollment in public school systems. Exceptional children, 81(1), 96-106.

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Disproportionality and Autism



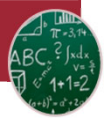
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Legal-Historical Context

Disproportionality in the Individuals with Disabilities Education Act (IDEA):

1997
- States required to define "significant disproportionality" (i.e., over-representation) and identify districts accordingly



2004
- Sig Dispro→15% federal funds shifted to comprehensive early intervening
- Review policies, procedures, practice



2016
- Equity in IDEA; Standard approach + state definition of thresholds
- Identification of "root causes"



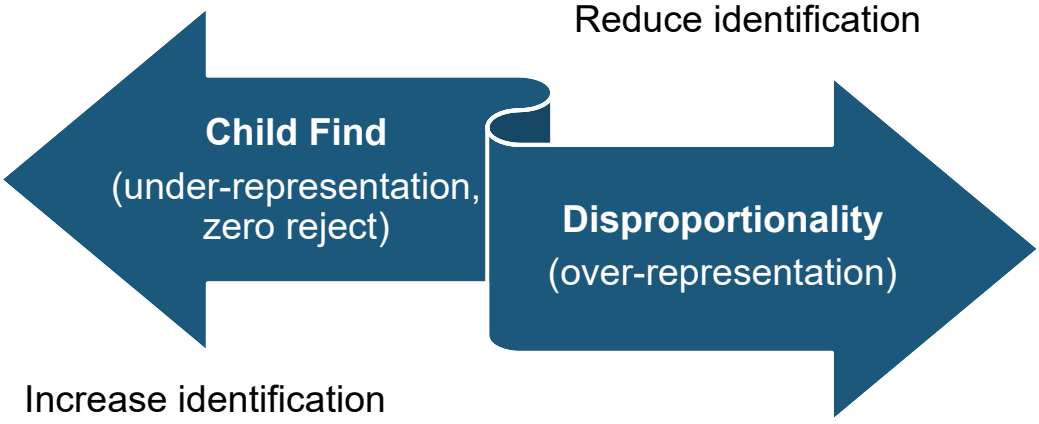
2018/19
- DeVos defers new rule
- Judge stays deferral
- DoJ appeals
- To be continued…



Sullivan, A. L., & Osher, D. (2019). IDEA's double bind: A synthesis of disproportionality policy interpretations. *Exceptional Children, 85*, 395-412. doi:10.1177/0014402918818047

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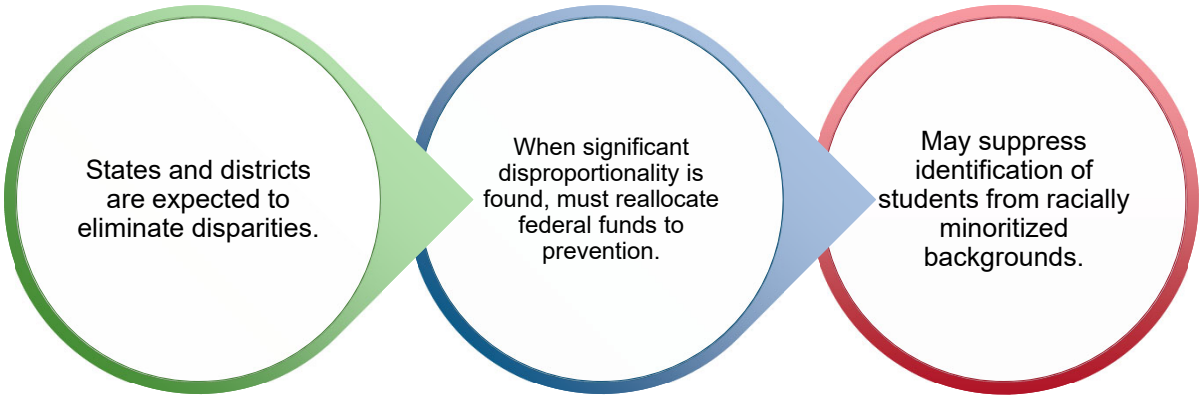
Disproportionality in Identification



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Why Does Disproportionality Policy Matter for ASD?



Sullivan, A. L., & Osher, D. (2019). IDEA's double bind: A synthesis of disproportionality policy interpretations. *Exceptional Children, 85*, 395-412. doi:10.1177/0014402918818047

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Mollie Weeks, MA, University of Minnesota

Case Law Shaping Autism Identification in Schools (preliminary results)

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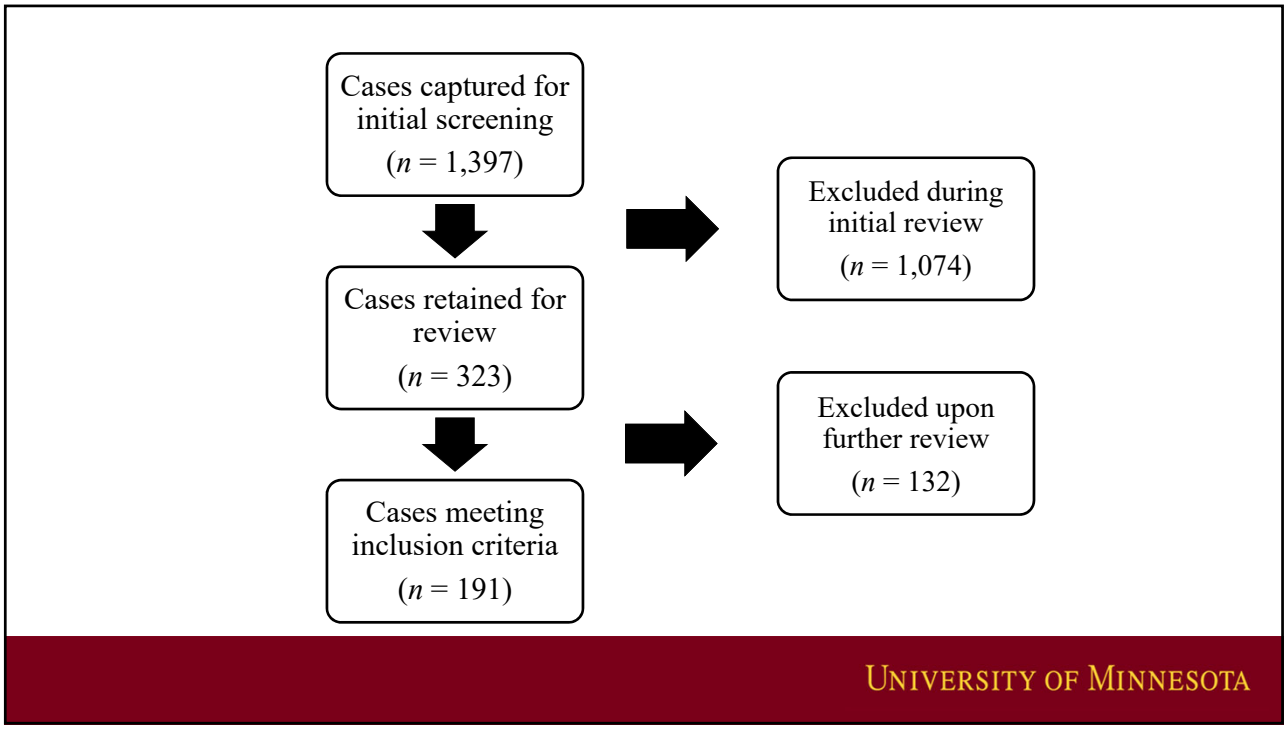
Study Purpose

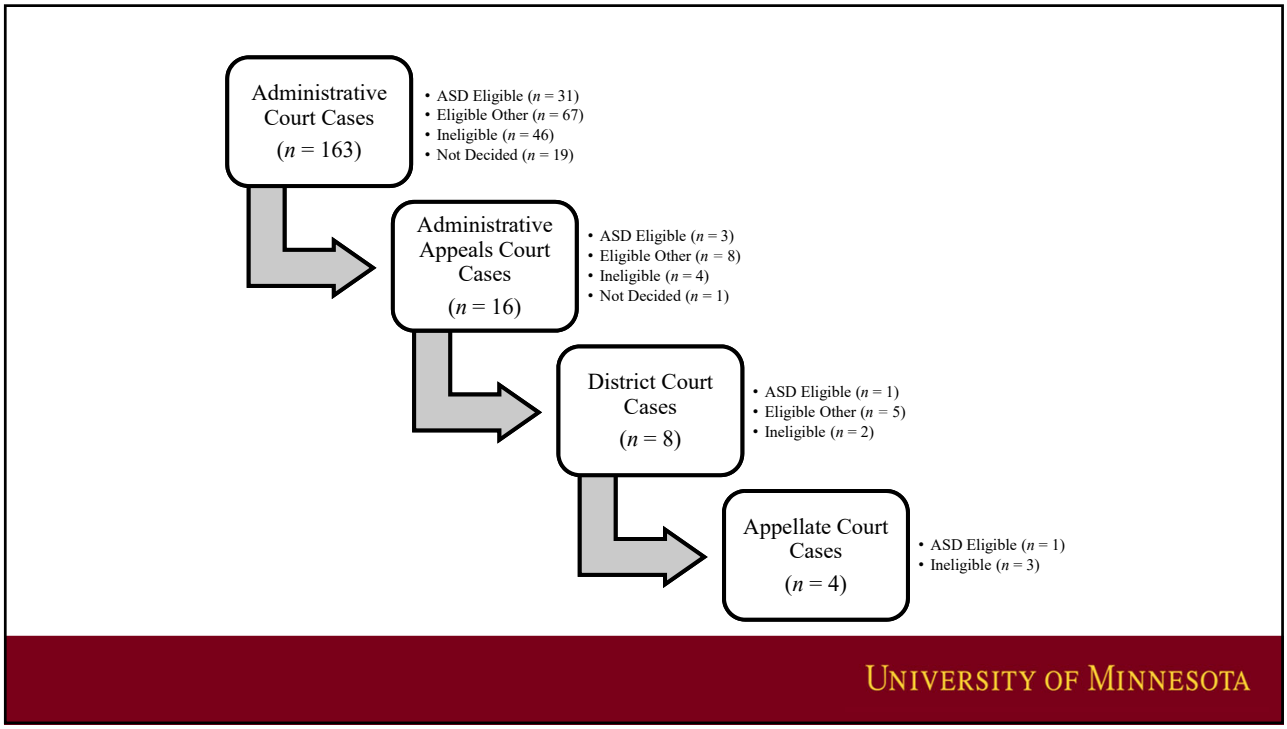
- Little attention is paid to issues of ASD identification within the legal system.
- Few studies using case law reviews to inform educational practices.
- Study Purpose:
  - Describe the status of ASD case outcomes and distill implications for families, educational policy, and educational practice.

Method

- Database: LRP Special Ed Connection
- Search Terms: ((autis%, eligib%), 5)
- Sequential Years: 1990-2016
- Exclusion Criteria:
  - Lower court decision or duplicate case;
  - Unrelated to IDEA;
  - Unrelated to eligibility or ASD; or,
  - Other reasons (e.g., OSEP policy memoranda)

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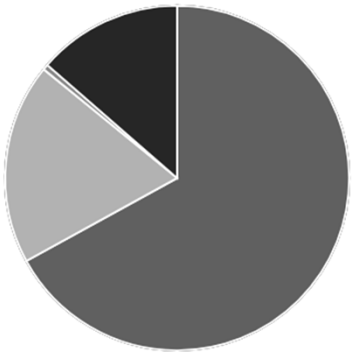




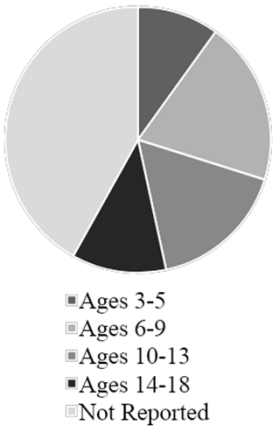
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Demographics

Sex



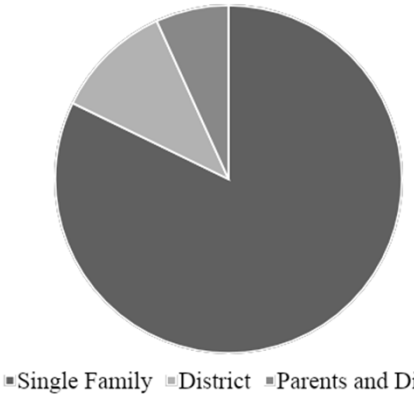
Age



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Plaintiffs and Decisions

Plaintiffs

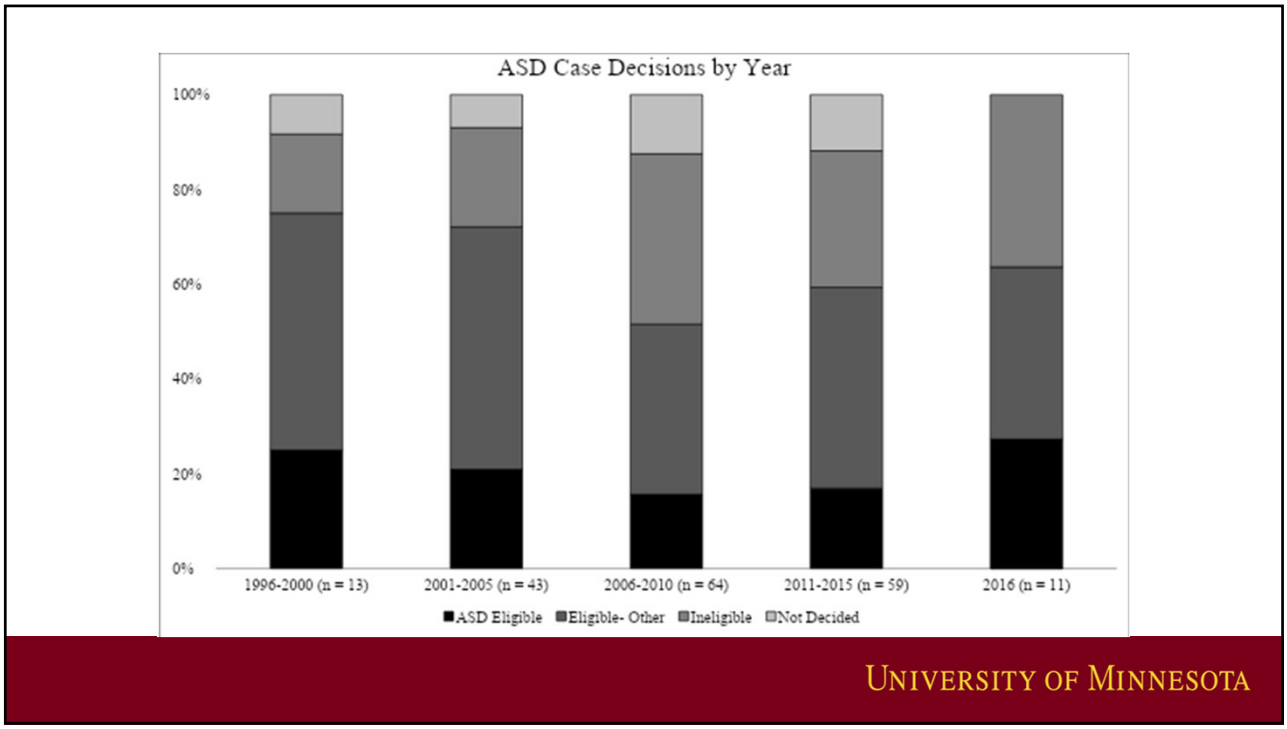


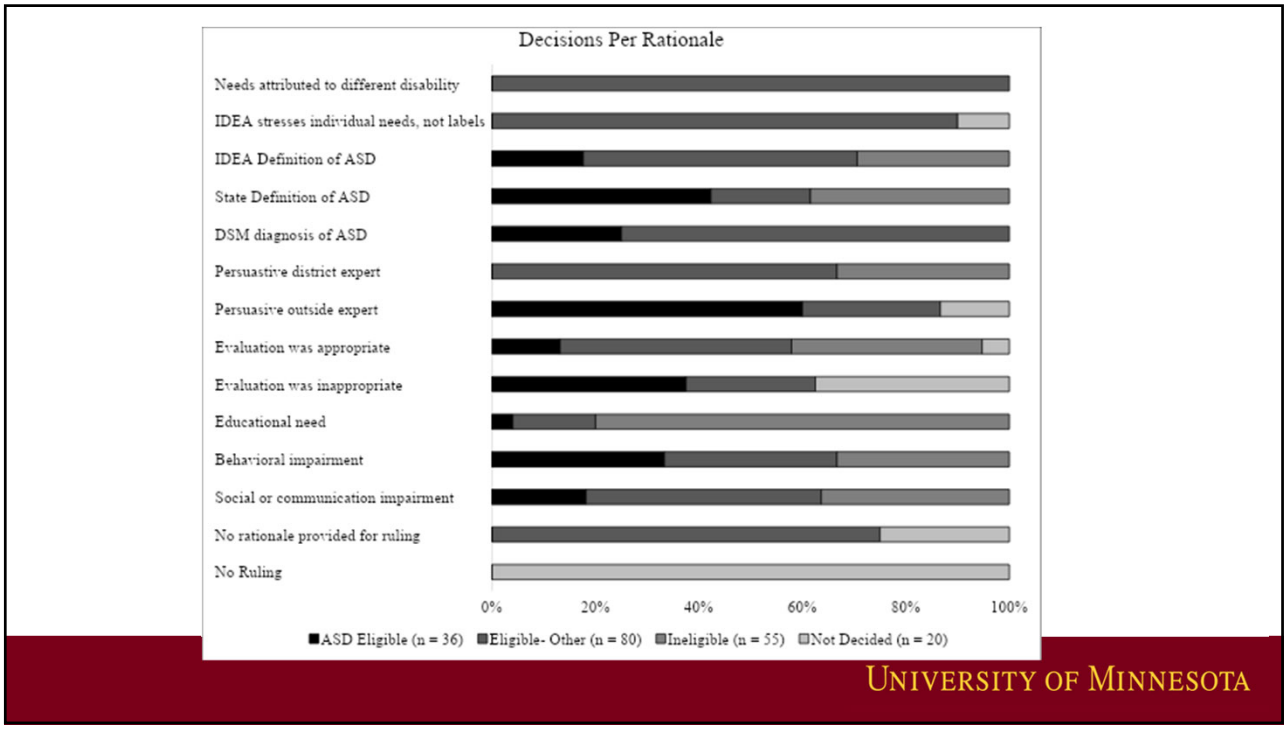
Decision



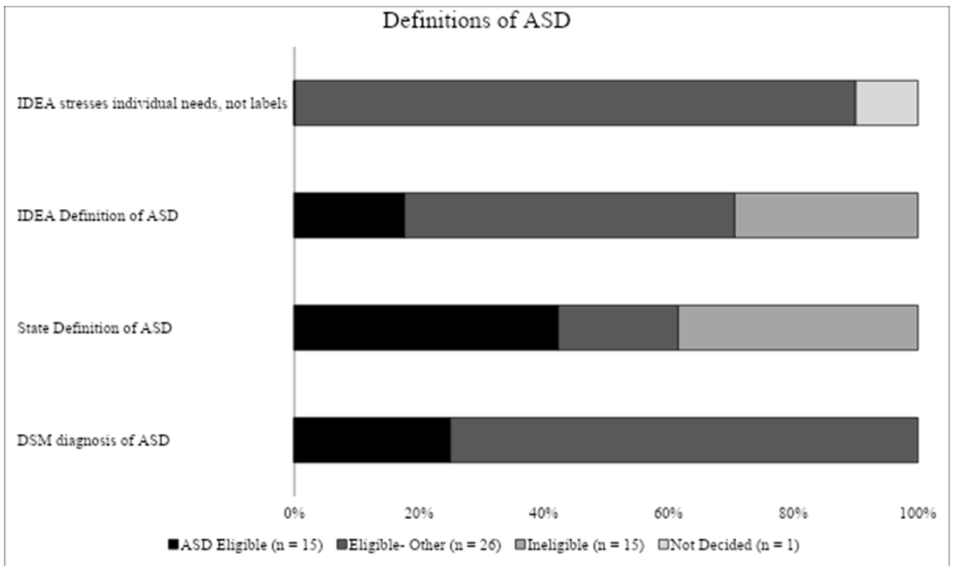
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Sullivan, A. L., Harris, B., Kulkarni, T., & Weeks, M. (2020, February). *Demystifying disproportionality in autism identification: Prevalence, policy, and best practice*. Symposium at the annual convention of the National Association of School Psychologists, Baltimore, MD.





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Case Examples

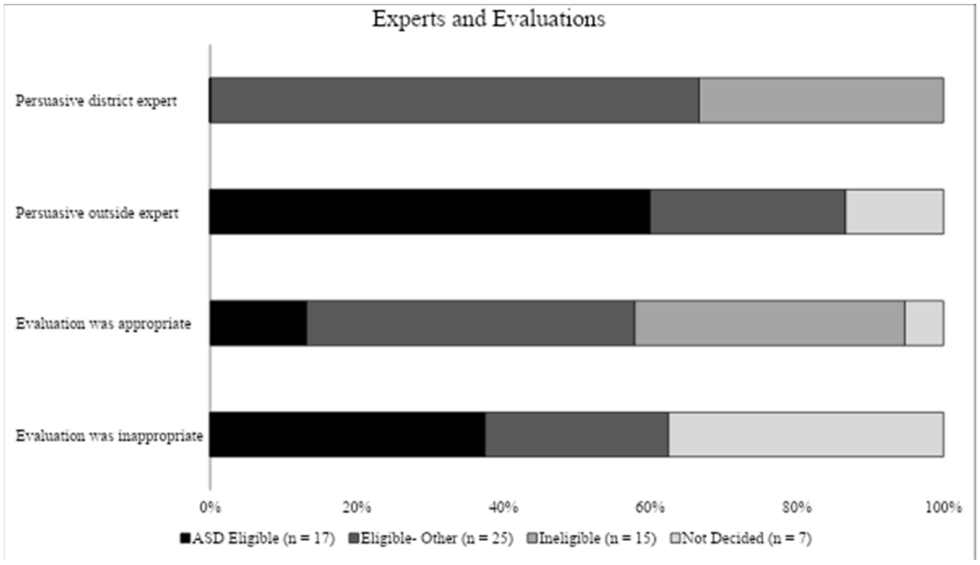
Needs not Labels

- Banquette Independent School District (2011)- family brought a case in which they claimed ASD eligibility was improperly removed. Judge ruled, based on testimony, that the student's ASD-like behaviors did not interfere with her ability to benefit from education; thus, qualifying her for SLD and SLI.
- Keller Independent School District (2012)- parents contended that removal of ASD label for ED was inappropriate. Judge ruled that, considering student's mental health diagnoses and social skills services on the IEP, ED was an appropriate label.

Considers labels to be a procedural issue which may not amount to a substantive violation of FAPE.

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Case Examples

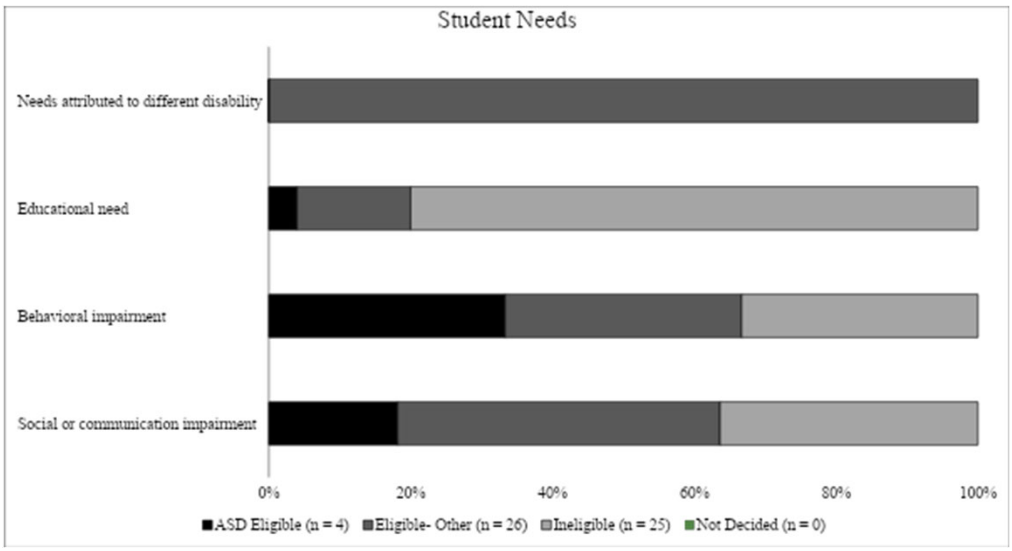
Evaluation Deemed Appropriate

- Berkley County School District- issue stated that the Hearing Officer erred when ruling that the student qualified for ASD (outside expert testified otherwise). Judge ruled that the school psychologist and evaluation were credible in determining eligibility.

Evaluation Deemed Inappropriate

- Lancaster Elementary School District (2007)- ordered to develop and conduct an additional comprehensive evaluation for several disability categories (including ASD) as their previous evaluation did not assess the student in all areas of suspected disability.

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Case Examples

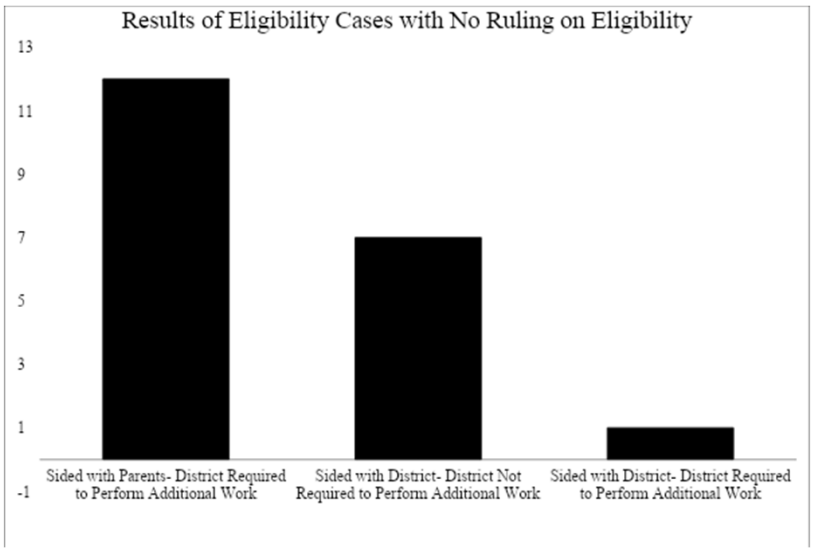
Social Impairment

- District of Columbia Public Schools (2016)- issue was whether a boy was denied FAPE as he was not found eligible for services under DD or ASD. Court ruled that his social skills were sufficient and, thus, qualified for special education services under another category.

Communication Impairment

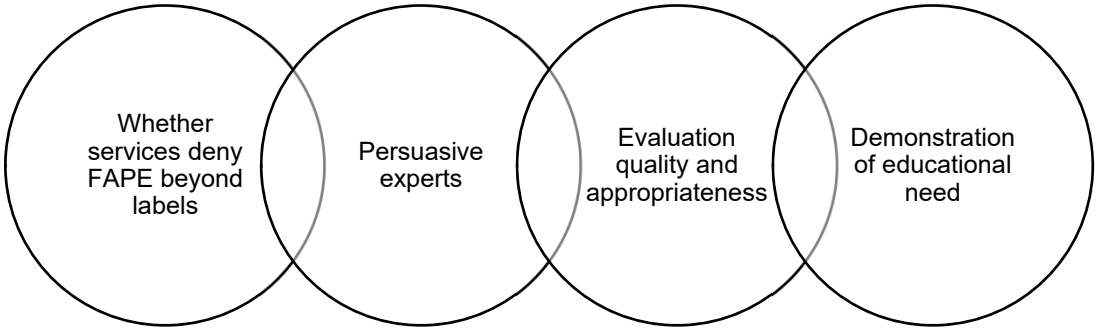
- Adelanto Elementary School District (2008)- issue was whether a boy was denied FAPE as he was not found eligible for services under ED, ASD, SLD, or SLI. Ruled that language deficits were due to SLD or SLI.

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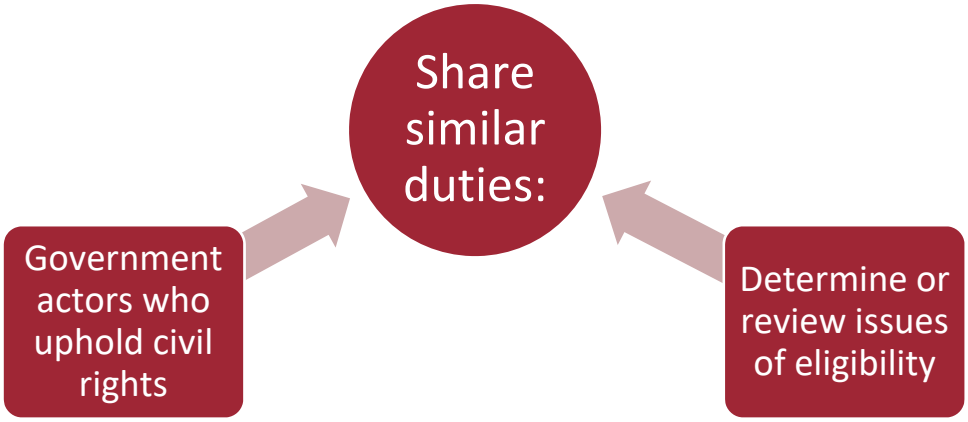
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What Matters in Due Process Decisions?



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School Psychologists and Administrative Law Judges



(Sadeh & Sullivan, 2017)

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Dual Problems: Ambiguity and Causation

Ambiguity	Causation
• Laws can have multiple interpretations • Challenges with determining educational performance and need • High ambiguity in eligibility decisions and evaluations • Ambiguity makes space for error in decision-making	• Determination of causation in eligibility and due process • Science avoids causal language • Cannot determine causation from tests, rating scales, etc. • If we cannot infer causation, how can we practice ethically?

(Sadeh & Sullivan, 2017)

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What to do?

- Place decisions within an ecological context
- Be vigilant for points vulnerable to bias in decision-making
- Problem-solving with low-inference decisions and malleable factors
- Consider disconfirmatory data
- Systematic decision-making rules and procedures

(Sadeh & Sullivan, 2017)

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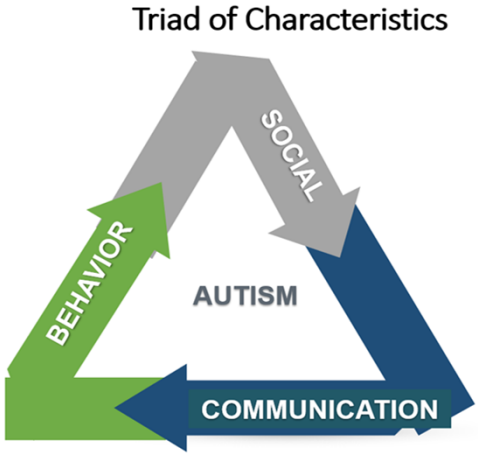
Bryn Harris, PhD, University of Colorado Denver

Culturally and Linguistically Appropriate Assessment

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The "Triad" of ASD

- What are we looking for?
  a. Lack of Communicative Competence
     - Ability to communicate in a fluid and integrated manner
  b. Impairments in Social Reciprocity
     - Ability to have back-and-forth social interactions with a variety of people
  c. Restricted, Repetitive Patterns of Behavior, Interests or Activities
     - Insistence on routines
     - Repetitive motor behaviors (e.g., hand-flapping, vocalizations)



Cultural and Language Considerations

"*While autism itself, the neuropathology of it, may not be culturally determined, our determination of those behaviours, and our interpretation of those behaviours is*"

- Dr. Mandell (CHOP)

Research Pertaining to Cultural Influences of Behavior

- Zhang et al. (2006) noted that eye contact with adults and pointing with the index finger is considered inappropriate behaviors within the Chinese culture
- Norbury and Sparks (2013) note potential cultural differences surrounding pretend play, public displays of emotion, and the extent to which children (especially boys) play with toy dolls
- These culturally imbedded behaviors and experiences will significantly impact the results of ASD assessments

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How do we evaluate for ASD in the school setting using culturally and linguistically responsive methods?

Sullivan, A. L., Harris, B., Kulkarni, T., & Weeks, M. (2020, February). *Demystifying disproportionality in autism identification: Prevalence, policy, and best practice*. Symposium at the annual convention of the National Association of School Psychologists, Baltimore, MD.

IEP Teams

- IEP teams should include:
  - Parents/Caregivers
  - General education teacher
  - Special Education teacher
  - Speech Language Pathologist
  - Other school professionals as appropriate (e.g. occupational therapist, social worker, English language acquisition teacher)
  - Assessment interpreter (e.g., School Psychologist)
  - Student - when appropriate
  - Knowledge of Autism Spectrum Disorder!

Potential Components of Autism Assessment

- Record Review
- Interview with School Staff
- Interview with Child
- Parent interview
  - Medical history
  - Developmental/social history
  - Adaptive behavior
- Rating scales
  - Multiple informants/settings
  - Range of functioning
- Observations
  - Structured
  - Semi-structured/Unstructured
- Standardized testing
  - Cognitive/Developmental
  - Language/Communication
  - Academic
  - Executive function
  - ASD Specific Measures
  - Social Skills

Parent Interview

- Most critical component for CLD children!
- Child cannot have ASD symptoms only at school or in one language
- Learn about family expectations for communication
- Language history
- Cultural factors (immigrant, refugee, other family members living at home etc.)
- Considerations when working with an interpreter

Recommendations for Observations (Dennison et al., 2019)

- When conducting observations, consider:
  - The difference between the student's skills communicating in the L1 and in English
  - Looking for symptoms of ASD that are not characteristic of a student acquiring a second language (e.g., lack of response to name, sensitivity to sensory stimuli, stereotypic behaviors)
  - The frequency and quality of attempts at social overtures, whether verbal or nonverbal
- Observations in different settings (e.g. classroom, PE class, recess, lunch)
- Conduct joint observations with another professional

Harris et al. (2014) ASD Assessment Study

- Authors conducted a comprehensive review of ASD assessments pertaining to culturally and linguistically responsive psychometrics and design
- Diagnostic tools assessed: ADOS, ADI-R, ASRS, CARS-2
- Screening tools assessed: ASQ:SE, ASSQ, GARS, M-CHAT, Q-CHAT, SCQ
- Findings: ASD screening tools had higher cultural and linguistic responsiveness than ASD diagnostic tools, practitioners need stronger tools for CLD populations

Reviewing the Psychometrics of an Assessment

- Theoretical basis and empirical information
- Standardization, reliability, and validity will be reported
- Standardization:
  - Norm sample (does it match or represent the intended population?)
- Reliability:
  - Evidence of score consistency
  - For example, test-retest reliability
  - Internal consistency reliability (high scores are generally above .80)
- Validity:
  - Adequacy of measuring the intended construct
  - Differential item functioning
  - Concurrent validity (such as giving scores on a similar ASD rating scale)

Psychometrics of ASD Assessments

- Mainly standardized with white, middle to upper class males
- Small samples of racially/culturally diverse children
- All Spanish language ASD diagnostic assessments are direct translations, not standardized
- What about other languages?
- Extremely limited research in this area

Psychometrics of Common ASD Tools

ASD Assessments Analyzed	Sample Size	African American	Latinx	White
Autism Spectrum Rating Scale (ASRS)-Short & Long Version for Teachers 2-5/6-18	1631	14.9	16.1	59.2
Autism Spectrum Rating Scale (ASRS)-Short & Long Version for Parents 2-5/6-18	1624	14.8	15.6	61
Gilliam Autism Rating Scale (GARS-3)	1107	12	10	80
Gilliam Autism Rating Scale (GARS-3)- Spanish	100	0	100	0
Social Responsiveness Scale (SRS-2)- School Age 4-18	1014	15.8	16.6	59.5
Social Responsiveness Scale (SRS-2)- Pre-School 2.5-4.5	247	14.2	16.6	64.4
Social Responsiveness Scale (SRS-2)- Adult	702	14.4	13.8	69.4
Social Responsiveness Scale (SRS-2)- Adult Self Report	702	14.4	13.8	69.4
Childhood Autism Rating Scales (CARS-2)- Standard Version	1034	16.2	12.7	60.5
Childhood Autism Rating Scales (CARS-2)- High Functioning Version	994	13.9	6.5	72.8
Childhood Autism Rating Scales (CARS-2)- Questionaire for Parents/Caregivers	Not Reported	Not Reported	Not Reported	Not Reported
Autism Diagnostic Interview-Revised (ADI-R)	50	Not Reported	Not Reported	Not Reported
Checklist for Autism Spectrum Disorder (CASD)	2469	Not Reported	Not Reported	Not Reported
The Autism Diagnostic Observation Schedule ADOS-2 Module 1-4	1619	11	4	80

Schmidt, J., Harris, B., Russell, C., Benallie, K, & McClain, M. (February, 2020). *Evaluating Psychometric Properties of Common ASD Identification Tools.* Poster presented at the National Association of School Psychologists Annual Conference.

Sullivan, A. L., Harris, B., Kulkarni, T., & Weeks, M. (2020, February). *Demystifying disproportionality in autism identification: Prevalence, policy, and best practice*. Symposium at the annual convention of the National Association of School Psychologists, Baltimore, MD.

Included in Validation

Not Included in Validation

ASD Assessments Analyzed	Reliability				Validity		
	Internal Consistency	Test-Re-Test	Inter-rater	Split-Half	Contruct Validity	Criterion Validty	Structural Validity
Autism Spectrum Rating Scale (ASRS)-Short Version for Teachers 2-5							
Autism Spectrum Rating Scale (ASRS)-Short Version for Parents 2-5							
Autism Spectrum Rating Scale (ASRS)-Short Version for Parents 6-18							
Autism Spectrum Rating Scale (ASRS)-Short Version for Teachers 6-18							
Autism Spectrum Rating Scale (ASRS)-Long Version for Teachers 2-5							
Autism Spectrum Rating Scale (ASRS)-Long Version for Parents 2-5							
Autism Spectrum Rating Scale (ASRS)-Long Version for Parents 6-18							
Autism Spectrum Rating Scale (ASRS)-Long Version for Teachers 6-18							
Gilliam Autism Rating Scale (GARS-3)							
Gilliam Autism Rating Scale (GARS-3)- Spanish							
Social Responsiveness Scale (SRS-2)- School Age 4-18							
Social Responsiveness Scale (SRS-2)- Pre-School 2.5-4.5							
Social Responsiveness Scale (SRS-2)- Adult							
Social Responsiveness Scale (SRS-2)- Adult Self Report							
Childhood Autism Rating Scales (CARS-2)- Standard Version							
Childhood Autism Rating Scales (CARS-2)- High Functioning Version							
Childhood Autism Rating Scales (CARS-2)- Questionaire for Parents/Caregivers							
Autism Diagnostic Interview-Revised (ADI-R)							
Checklist for Autism Spectrum Disorder (CASD)							
The Autism Diagnostic Observation Schedule ADOS-2 Module 1							
The Autism Diagnostic Observation Schedule ADOS-2 Module 2							
The Autism Diagnostic Observation Schedule ADOS-2 Module 3							
The Autism Diagnostic Observation Schedule ADOS-2 Module 4							

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Selected ASD Specific Assessments - Most Commonly Used in Educational Evaluations

Childhood Autism Rating Scale, 2nd Edition (CARS-2)

Gilliam Autism Rating Scale, Third Edition (GARS-3)

Checklist for Autism Spectrum Disorder (CASD)

Autism Diagnostic Interview- Revised (ADI-R)

Autism Spectrum Rating Scales (ASRS)

Social Responsiveness Scale (SRS-2)

Autism Diagnostic Observation Schedule (ADOS -2)

- Consider individual child, culture/language considerations, psychometrics, what assessments other team members are using, usefulness for intervention

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Limitations of the ADOS-2 in School Settings

- Training, expense, logistics with extensive testing materials, limitations of ASD teams that travel to do ASD assessment, limited research of ADOS with diverse cultural groups, not required for educational identification of ASD

Problematic Practices – ASD Assessment

- Not using the body of evidence
  - Weighting one tool/measure heavily
- Not adequately including families
- Not collaborating with multidisciplinary team members and integrating data
- Not involving ELA staff
- Using broad band screening measures solely (e.g. BASC)
- Building mental health staff not involved in assessment
- Not questioning the impact of culture and language throughout assessment
- Use of C-LIM (e.g., Styck & Watkins, 2013)

Problematic Practices – Assessment (cont.)

- Not considering other educational identification categories or health conditions (e.g. intellectual disability, SED, OHI, Anxiety)
- Limited or no consideration of trauma
- Retaining Developmental Disability eligibility until age 8
- Concerns of ASD in early childhood, although no ASD specific measures were administered
- No collaboration/communication with outside providers
- Requiring MTSS process prior to an ASD evaluation
- Not understanding the family and their expectations for their child as well as their knowledge of ASD

What can we do? Many things!

- Evaluate our own school and district data!
- Strong tier 1 and 2 practices
- Strong parent communication/home-school collaboration
- Good transition services between Part C and Part B (and preschool to kinder)
- MTSS Process (progress monitoring data points not required for an ASD eligibility)
  - What data should we be gathering?
  - Who should we involve?
- Special Education Evaluation/Services
  - Types of assessments selected
  - Interpretation of assessments
  - Culturally and linguistically responsive interventions

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Practices to Promote – ASD Assessment

- Family interview! Developmental history is key!
  - This may occur with an interpreter

Practices to Promote (cont.)

- Obtain more qualitative information (more interviews, observations, comparisons to other like-bilingual peers, record reviews)

Practices to Promote (cont.)

- Consult with ELD teachers and SLP regarding the impact of language acquisition

Practices to Promote (cont.)

- If conducting standardized measures, take extensive notes about current areas of functioning and need. Paint a picture of that child's current functioning in the report – much more useful than a number for intervention purposes
  - Standardized scores are not required for an educational identification of ASD, evidence of educational impact is

Practices to Promote (cont.)

- Rely on a body of evidence – one assessment should never ”trump” other data

Practices to Promote (cont.)

- Do not wait to assess the child so they can obtain more English language proficiency

Practices to Promote (cont.)

- Utilize community resources (faith-based organizations, family services agencies, community partners with strong connections to the families to advocate for early identification)

Practices to Promote (cont.)

- Advocate for more assessments that have strong psychometrics with racially/ethnically and linguistically diverse children

Practices to Promote (cont.)

- All decisions about eligibility are made as a team. Arrange meetings with the team prior to the IEP to review data and consider if there are holes in the information gathered

Practices to Promote (cont.)

- As we currently don't have optimal standardized ASD assessments for CLD populations, obtain more qualitative data and collaborate with the team in a more purposeful way

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Thank you for joining us!

Questions?

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